

SUMMARY

DECISION NO. 725/96

Accident (occurrence).

The worker appealed a decision of the Hearings Officer denying entitlement for a low back injury which the worker claimed he suffered in an unwitnessed accident at work when he slipped on ice and fell, striking his back on a door jamb.

The worker completed his shift. Later that day, the worker experienced severe back pain at home while lifting a box of kindling wood. The Board found that the worker's condition was related to the incident at home.

The worker was credible. The Panel accepted the worker's explanation for not telling co-workers of the fall, that he was afraid of losing his job. The worker was consistent in telling all health care professionals that he fell at work but only started experiencing pain later at home while lifting the wood.

The Panel concluded that the worker fell at work as claimed and that he injured his back in the fall. There was medical opinion that the worker suffered the injury in the fall but that the inflammatory process took a number of hours to set in.

The appeal was allowed. [9 pages]

PANEL: Sutherland; Crocker; Séguin

DATE: 28/08/96

WORKERS' COMPENSATION APPEALS TRIBUNAL

DECISION NO. 725/96

This appeal was heard on August 16, 1996, by a Tribunal Panel consisting of:

S.J. Sutherland: Vice-Chair,
J. Seguin : Member representative of employers,
J.A. Crocker : Member representative of workers.

THE APPEAL PROCEEDINGS

The worker appealed the decision of the Workers' Compensation Board Hearings Officer, A. LaCivita, dated March 17, 1995. In this decision, the Hearings Officer denied the worker initial entitlement to benefits for a low back injury which the worker claimed was causally related to an accident that arose out of and in the course of his employment on October 19, 1992.

The worker appeared and was represented by H. Conroy, from the Office of the Worker Adviser. The employer was informed of the proceedings and advised the Tribunal on August 3, 1995, that they would not be participating.

The worker's wife observed the proceedings.

THE EVIDENCE

The Panel had before it the Case Description materials prepared by the Tribunal Counsel Office, together with three Addenda. These were marked as Exhibits #1 to #4, respectively. When she made her submissions, Ms. Conroy provided the Panel with an excerpt from *Personal Injury, a medico-legal guide to the spine and limbs* by D.J. Ogilvie-Harris and G.J. Lloyd. This excerpt was marked as Exhibit #5.

Ms. Conroy made a brief opening statement and closing submissions. The worker gave evidence under oath.

THE NATURE OF THE CASE

The worker claimed that on October 19, 1992, he slipped on ice while carrying two or three boxes of oil. He fell, striking his back on a door jamb. There were no witnesses to the accident. The worker completed his shift without telling anyone he had had an accident. Later that day, while he was lifting a box of kindling wood at home, he experienced severe back pain. This pain continued and worsened over the next 24 hours. He returned to work the day after the accident and at the end of his shift, went to the Emergency Department at the local hospital where he was treated and admitted.

The Board adjudicators found that the worker's back problems were related to the incident at home and not to the fall at work, and denied him benefits. The Hearings Officer confirmed that denial and concluded, in addition, that the worker had not established beyond a reasonable doubt, that he had an accident at work on October 19, 1992.

The Panel must determine, on a balance of probabilities, whether the worker had an accident at work on October 19, 1992, and whether his ongoing back problems resulted from that accident.

THE PANEL'S REASONS

(i) The worker's testimony

The worker testified that in October 1992, he had been working for the accident employer for about four months. His work included pumping gasoline, serving customers, bringing stock in from a shed, filling shelves, washing floors, cleaning the yard, and sweeping floors. He said that he had a bad shoulder, and the job required using it more than he ought, but other than this, he had no problem doing his work.

On October 19, 1992, the station was quite busy. The mechanic did not have oil in the shop and the worker had to bring it in from the shed, as well as wait on customers. The mechanic urged him to hurry up. The worker said that he went to the shed, and picked up cases of oil (he could not recall how many, but knew there were more than one). He slipped and fell, his lower back hitting the door jamb. He stated that he lay on the floor, in severe pain, for some time. The mechanic called him, but did not come to see what was wrong.

The worker said that as soon as he was able to do so, he got up and walked around a bit. He did not take the oil to the mechanic at that point. He waited on customers, then went back for the oil.

The worker testified that he carried Tylenol #3 with him because of his shoulder pain, and took it as required. After he hurt his back in the fall, he started taking the Tylenol #3 to overcome the back pain. He stated that he did not want to lose his job so he finished the shift. However, the pain continually got worse as the day went on.

When the worker was at home that evening, he went outside to get some kindling wood. He said he was walking up a hill, carrying the box of wood, when he started to feel shaky. This shakiness caused him to fall. He could not remember whether he fell to one knee, or onto both of them. He testified that the shakiness had occurred during the day as well, but he had been taking Tylenol #3, and felt that the medication had covered it up.

The worker stated that the pain got worse through the evening. He took more medication and went to bed. When he woke up in the morning, he could hardly move because the pain was so severe. He rolled out of bed and went to work in a great deal of pain. He testified that on this day, he told the mechanic about his accident the previous day. The mechanic told him that the station owner fired employees if they got hurt and warned him to be careful what he said to the owner.

The worker said that during the day, his legs felt as though there was lightning going down them and into his feet. In addition, they kept getting numb. His wife came to his workplace at about 11:30 a.m. and recommended that he go to the hospital. He said that he told her he would take more pills and complete his shift because he did not want to lose his job. He testified that he remembered crying at lunch time, with his back pain. At 4:00 p.m., he went to the hospital. By that time, he was nearly screaming with the pain. He was treated right away

and immediately given Demerol to alleviate the pain. When the first injection of Demerol did not control the pain, he was given another about 45 minutes later.

Ms. Conroy directed the worker's attention to the Emergency Department Report which said the following:

30 year old - fell yesterday onto back - 0 hurt. Then last night -
lifting box - gradually suffered pain in back -> today v. bad.

The worker said that he wanted to be honest about the fact that he had two accidents. He said that he told the doctor that he did not feel the back pain after the accident at work because he was taking Tylenol #3. Also, prior to giving this statement, he had been given two injections of Demerol and was not thinking clearly.

In response to questions from Ms. Conroy, the worker said that he had the Tylenol #3 because of a previous compensable injury in which he dislocated his left shoulder. He has had two operations but the shoulder has not returned to its pre-injury condition and he receives a 35% pension for this condition. His doctor prescribed Tylenol #3 for the ongoing shoulder pain and he takes them as needed. On October 19, 1992, he took many more Tylenol #3 than usual but he had no idea how many he had taken.

The worker testified that his back kept getting worse after he was discharged from the hospital.

(ii) Medical evidence

Most of the medical evidence in the file was related to the worker's ongoing low back condition. Little of it addressed whether he fell at work or whether the ongoing back problems were related to a workplace injury or to an incident at home. The matter the Panel must determine is whether an accident happened at work as the worker claimed, and if it did, whether it made a significant contribution to his ongoing low back problems. Therefore, the Panel will only review the medical evidence that casts light on this issue.

The Emergency Department Report quoted above had a stamp on it that said: "this visit was related to an injury at work." The box indicating "no" was marked and appears to have been initialled by the nurse. The worker's family physician, Dr. A. Knight wrote "Prob WCB", close to the bottom of the page.

When the worker was discharged from hospital on October 26, 1992, Dr. Knight wrote a discharge report. The summary described the onset of the worker's back pain. It said:

This 30 year old man fell the day prior to admission, landing flat on his back. Initially he was not hurt. However, later that evening he gradually developed pain in his back after lifting a box. The following day the pain was much more severe and he was complaining of getting back spasms. There was no radiation of pain to his legs.

The admission diagnosis as reported by Dr. Knight, was acute low back pain. The discharge diagnosis was back strain.

The Doctor's First Report was completed by Dr. Knight on October 31, 1992. Dr. Knight indicated that the worker had "slipped at work, falling backwards, landing on back across piece of wood".

The worker returned to the Emergency Department at the hospital, on October 31, 1992, with back pain. He was treated and released. The stamp on this report indicated that the injury occurred at work.

The worker consulted Dr. J.A. Mayer, a neurologist, and Dr. M. Kleinman, a consultant in physical medicine, on December 9, 1993, and May 15, 1995, respectively. The accident history recounted by each of these doctors in their reports was the same as the worker described to the Panel on the day of the hearing.

Ms. Conroy wrote Dr. B. Alpert, an orthopaedic surgeon with a special interest in head, neck, and back pain, on May 24, 1996. She provided him with the worker's description of both the events that happened on October 19, 1992, and asked:

Of the three incidents -- the fall on the door jamb, lifting the box of kindling, slipping onto one or both knees, which in your opinion is likely to have caused the symptoms which manifested the next morning? Which of the three, if any, is likely responsible for the current findings and symptoms?

In her letter, Ms. Conroy asked Dr. Alpert to provide a detailed explanation of the mechanics involved.

Dr. Alpert provided a lengthy response to Ms. Conroy's letter, on July 9, 1996. He stated that he had been treating the worker since November 20, 1995, for his chronic low back injury. He had reviewed reports from ten specialists, including neurosurgeons, vascular surgeons, and orthopaedic surgeons, along with radiology records from three hospitals (he provided the names and the areas of specialization of each of the doctors and the names of the hospitals, in his report). He recounted the accident history as given to him by the worker, by Ms. Conroy, and as recorded in the documentary materials before him. There were no significant differences between this history and the description of events that the worker provided for the Panel in his testimony at the hearing.

Dr. Alpert discussed in some detail, the reports and opinions of other doctors, as well as the reports pertaining to the various tests that had been done on the worker (CAT scans and MRI's). He described the results of own physical examination of the worker. His diagnostic impression was:

Chronic lumbar musculoligamentous strain and facet syndrome, with post-traumatic accelerated degenerative disc disease at L5/S1 disc, leading to chronic mechanical and some radicular lower back pain, primarily due to work related injury of October 19, 1992.

The "Discussion" section of Dr. Alpert's report was particularly helpful to the Panel, and although it was lengthy, we have reproduced it in total, below.

As noted above, the most significant injury to this patient's lumbar spine appears to have involved the injury at work on

October 19, 1992, when he was working at a service station. On that date, the patient was apparently leaving a shed, when he slipped on the ice and fell onto his lower back just above the buttock region, apparently landing on the door jamb and having two cases of oil fall on him. Injury would have likely involved acceleration-deceleration axial loading with flexion and rotation of the spine, which has been shown in studies to lead to the greatest amount of intravertebral disc loading. The intravertebral discs act as the shock absorbers of the spine. The supporting muscles and ligaments are the secondary shock absorbers. As the internal disc pressure is increased and the outer fibres of the disc, known as the annulus fibrosis, undergo over stretching with tearing and shearing, the disc contents are able to bulge and even go on to frank herniation in some cases. It is entirely plausible that the degenerative disc disease process can be initiated or accelerated by the above injury which would tend to damage the intradiscal substance material as well as the supporting walls of the disc. The supporting muscles and ligaments of the lumbar spine would have been overstretched and partially torn with deep contusion as a result of the above injury. The inflammatory process, however, is not always immediate and often takes anywhere from six to twelve hours or even up to twenty-four or forty-eight hours to set in, as microscopic inflammatory changes eventually build up and lead to gross changes. This includes local capillary hemorrhage, with inflammatory exudate and edema, as well as the development of subsequent irritative and protective muscle spasm. The sciatic nerves may have been bruised at the time of the above fall, given that the patient landed just above the buttock region where the upper portion of the sciatic nerve is just beginning to travel into the lower limb and likewise the roots of the sciatic nerves from L4 to S1 in particular are prone to being over-stretched and inflamed. Therefore, there does not always have to be frank, persisting neural impingement on the sciatic nerves to have a component of radicular pain which may be related to the actual sciatic nerve injury with subsequent stretching and scarring into the nerve as it undergoes healing as well as from referred pain due to the mechanically irritated and persistently strained facet joints. Although approximately 90 to 95% of cases go on to have good resolution of the acute injury and inflammation, approximately 5 to 10% of cases develop chronic inflammatory scar tissue healing with recurrent and persistent pain, due to the weakened and irritated support structures in the lumbar spine. It should be noted that the initial x-rays of the lumbar spine performed following the above injuries were read as being within normal limits but within approximately three and a half years, in this relatively young, 34 year old patient, there is already evidence of degenerative disc

disease on both plane x-rays as well as further damage seen on MRI scan, consistent with the mechanism of injury described above. I have already noted in this report earlier that Dr. Mayer, neurosurgeon, who was following and treating this patient early on, also was of the opinion that the work-related injury of October 19, 1992, in which the patient slipped and fell at the service station, was the primary injury responsible for the patient's permanent impairment and permanent disability with inability to return to work.

Given the patient's history of trying to finish his work-shift on October 19, 1992, but noting some pain and stiffness in his lower back, it would appear that the inflammatory process was already commenced following the history of fairly significant work-related trauma noted. The patient does not appear to have suffered significant injury from lifting the box with kindling at his house on the evening of October 19, 1992, but rather had minor aggravation of the significant prior injury, which occurred at work. In fact, the patient noted that his lower back and legs started to give out on him as he attempted to carry the box of kindling, such that he felt some numbness to his legs, likely due to the sciatic stretch or referred radicular pain already noted and he did not sustain as violent a fall as earlier, in that he merely went down on one or both knees. There was further increasing discomfort felt through the evening and by the morning, which is the classic history for an injury from the day before of significance with subsequent inflammatory insult building up, the patient had difficulty getting out of bed and had to seek emergency care.

A CT-scan of the lumbar spine performed in January of 1993, showed evidence of disc injury, which at L5/S1 included a left lateral disc herniation and at L4/L5 a central bulge of the disc. A subsequent CT-scan performed in March of 1995, did not show evidence of disc herniation, which may relate to either technical limitations with respect to that CT-scan study, and that MRI scanning is more accurate, as well as the fact that certain sized bulges and herniations can undergo some element of healing. However, I have noted that MRI scanning is much more sensitive and thorough in its ability to detect disc damage as compared to CT-scanning. A recent MRI-scan performed in March of 1996, has been noted to show degenerative disc changes at L5/S1 with decreased signal intensity within the disc and slight loss of disc space height as well as a right posterolateral disc bulge and degenerative changes at L4/L5 disc including decreased signal in the disc and some posterior osteophyte formation. No evidence of frank disc herniation or neural compression was visualized. Again, given this patient's relatively young age of 34 years, it is not likely

that absent the work-related injury of October 19, 1992, he would have the MRI scan findings described above. The MRI findings are most consistent with post-traumatic accelerated disc damage in the lumbar spine at L5/S1 and somewhat at L4/L5. I have included a copy of the MRI scan of the lumbar spine for your review, as requested.

Unfortunately, this patient has developed chronic pain syndrome as a result of his lower back injury, which tends to complicate his clinical course and treatment. However, there is clearly real evidence of physical pathology and injury in this patient's lumbar spine, which is most consistent with the mechanism of injury occurring on October 19, 1992, when he slipped and fell at work while employed as a service station attendant and this appears to be the most significant injury and causation of his condition. The above has also led to the permanent disability and inability to work. His prognosis remains poor to guarded.

(iii) The law and Board policy

Section 4(1) of the Workers' Compensation Act, R.S.O. 1990, c. W.11, provides that where personal injury by accident arising out of and in the course of employment is caused to a worker, the worker is entitled to benefits in the manner and to the extent provided by the Act. Tribunal cases have interpreted the test for causation to be whether the work accident is a significant contributing factor to the disability.

(iv) Conclusions

(a) Did the worker have an accident at work on October 19, 1992?

The Panel finds that the worker did have an accident at work on October 19, 1992. When the Hearings Officer concluded that the worker had not established the happening of an accident at work, he relied heavily on the fact that the worker did not tell his co-workers about the accident when it occurred. The Panel found the worker a credible witness and we accept his testimony that he was afraid of losing his job. We were also assisted in coming to the conclusion that there had been an accident at work, by the fact that the worker gave all the health care professionals who treated him essentially the same accident history, including the fact that his back started to hurt after lifting a box of kindling at home.

(b) Did the worker injure his back in the fall at work on October 19, 1992?

The Panel found Dr. Alpert's detailed analysis of great assistance in answering this question. Dr. Alpert knew about both incidents, i.e. the fall at work and the onset of pain at home after the worker lifted a box of kindling. He had examined and treated the worker and he had access to all the medical reports relating to the worker. He was able to explain to the Panel's satisfaction the mechanics of the workplace injury, how the fall caused the damage to the worker's lumbar spine that is now evident, and why the pain became more severe over the 36 or so hours between the fall and the worker seeking medical treatment. He also explained that lifting the kindling and/or falling to a knee or knees may have aggravated the earlier workplace injury but likely were not significant events in the development of the low back pain.

The Panel finds that the worker injured his lower back in a fall at work on October 19, 1992.

THE DECISION

The appeal is allowed. We leave it to the Board to determine the nature of the benefits to which the worker is entitled.

DATED: August 28, 1996

SIGNED: S.J. Sutherland, J. Seguin, J.A. Crocker